



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

MOCK 4 PAPER 1 TEST DATED ON 7th JUNE 2026

Q1. A 19-year-old boy develops fever, periorbital edema, chemosis, severe headache, diplopia, and loss of corneal reflex after squeezing a pustule on the upper lip. MRI venography shows septic thrombosis of a dural venous sinus lateral to the body of sphenoid. Examination shows lateral rectus palsy and ophthalmoplegia.

Which statement is INCORRECT?

- a) CN VI is more vulnerable because it lies freely within the cavernous sinus beside the internal carotid artery
- b) CN III, IV, V1, and V2 lie in the lateral wall of the cavernous sinus
- c) CN II is commonly damaged because it traverses the cavernous sinus along with the ophthalmic artery
- d) Infection may spread through facial and ophthalmic veins due to absence of valves

Q2. A 9-month-old boy is brought for developmental regression. He previously rolled over and sat with support but now has poor head control, exaggerated startle response to sound, and progressive hypotonia. Fundoscopy shows a cherry-red macula. There is no hepatosplenomegaly. His parents are of Ashkenazi Jewish ancestry. Which enzyme is most likely deficient?

- a) Acid sphingomyelinase
- b) Hexosaminidase A
- c) Glucocerebrosidase
- d) Arylsulfatase A

Q3. A 40-year-old male presents with difficulty while walking after a hip injury. During clinical examination, he is asked to stand on his left lower limb while lifting the opposite limb off the ground. The examiner observes that the pelvis drops on the right side instead of remaining level. There is no significant pain, but weakness is noted in hip stabilization. Neurological examination suggests involvement of a nerve supplying the hip abductors.

Which muscles are most likely paralyzed in this patient?

- a) Right gluteus medius & gluteus minimus
- b) Left gluteus medius & gluteus minimus
- c) Right gluteus maximus
- d) Left gluteus maximus

Q4. An 11-month-old infant presents with failure to thrive, progressive neurodevelopmental decline, hypotonia, recurrent respiratory infections, and a protuberant abdomen. Examination shows



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hepatosplenomegaly. Fundoscopy reveals a cherry-red spot. Bone marrow biopsy shows lipid-laden foam cells.

Which accumulated substrate best explains this disorder?

- a) GM2 ganglioside
- b) Glucocerebroside
- c) Sphingomyelin
- d) Ceramide trihexoside

Q5. A 32-year-old male is brought to the emergency department following a road traffic accident. He complains of inability to void urine and severe perineal pain. On examination, there is swelling of the perineum, scrotum, and penis, with progressive extension of edema to the lower anterior abdominal wall. Blood is noted at the urethral meatus. Digital rectal examination reveals a normal prostate position. Imaging suggests anterior urethral disruption with urine extravasation tracking along fascial planes.

Which part of the urethra is most likely injured in this patient?

- a) Prostatic urethra
- b) Membranous urethra
- c) Spongy urethra
- d) Bulbar urethra

Q6. A 66-year-old hypertensive man suddenly develops dense weakness of the left face, arm, and leg. Sensation is relatively preserved. There is no cortical aphasia, neglect, or visual field defect. CT angiography suggests rupture of small penetrating arteries supplying a compact white matter region. Which artery is most likely involved?

- a) Lenticulostriate branches of middle cerebral artery
- b) Anterior cerebral artery
- c) Posterior inferior cerebellar artery
- d) Anterior spinal artery

Q7. A tumor near the cerebellopontine angle compresses the lateral aperture of the fourth ventricle and causes noncommunicating hydrocephalus.

Which opening is obstructed?

- a) Foramen of Monro



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- b) Cerebral aqueduct
- c) Foramen of Luschka
- d) Foramen magnum

Q8. A 16-year-old girl with Gaucher disease type 1 has splenomegaly, anemia, thrombocytopenia, and bone pain but no primary neurologic disease. Her physician recommends enzyme replacement therapy. Why is enzyme replacement more useful in this patient than in classic Tay-Sachs disease?

- a) Glucocerebrosidase acts primarily in circulating plasma rather than lysosomes
- b) affects macrophage-rich organs accessible to infused enzyme
- c) Tay-Sachs is caused by mitochondrial rather than lysosomal dysfunction
- d) Hexosaminidase A cannot be synthesized by recombinant DNA technology

Q9. A researcher studies sphingolipid synthesis in neurons. The first committed step requires condensation of serine with a fatty acyl-CoA to produce a sphingoid base precursor. A toxic plant compound inhibits this initial reaction.

Which molecule donates the amino alcohol backbone for sphingolipid synthesis?

- a) Glycine
- b) Serine
- c) Alanine
- d) Choline

Q10. A 17-year-old boy is brought for evaluation after his father died suddenly from myocardial infarction at age 38. The patient has firm nodular swellings over the Achilles tendons and extensor tendons of the hands. Corneal arcus is present. Fasting lipid profile shows total cholesterol 410 mg/dL, LDL-C 335 mg/dL, triglycerides 110 mg/dL, and HDL-C 42 mg/dL. His mother has normal lipid levels, but his paternal grandfather died of premature coronary artery disease.

Which molecular defect most likely explains this patient's lipid abnormality?

- a) Defective lipoprotein lipase causing impaired chylomicron clearance
- b) Defective LDL receptor-mediated hepatic uptake of LDL particles
- c) Defective apolipoprotein C-II activation of lipoprotein lipase
- d) Increased hepatic VLDL production due to insulin resistance



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Q11. A 36-year-old man has progressive renal insufficiency, left ventricular hypertrophy, burning extremity pain, corneal verticillata, and clusters of angiokeratomas. His mother has mild proteinuria and neuropathic pain. Genetic testing confirms a lysosomal enzyme defect. Which stored lipid is most likely accumulating?

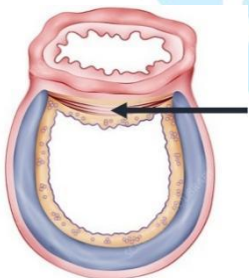
- a) GM2 ganglioside
- b) Sphingomyelin
- c) Glucocerebroside
- d) Ceramide trihexoside

Q12. A 55-year-old man undergoes upper gastrointestinal endoscopy for evaluation of progressive dysphagia. During the procedure, the endoscopist encounters a physiological constriction of the esophagus at approximately 23 cm from the upper incisor teeth. The narrowing is attributed to external compression by a major thoracic vascular structure. The clinician recalls that these anatomical constrictions are important sites for foreign body impaction and pathological strictures.

Which structure is responsible for this constriction?

- a) Arch of aorta
- b) Right bronchus
- c) Main bronchus
- d) Left bronchus

Q13. A histological transverse section of the upper thorax is shown, demonstrating the trachea anteriorly and esophagus posteriorly. The trachea is identified by its characteristic C-shaped hyaline cartilage rings, with a gap posteriorly. The arrow points to a band of smooth muscle bridging the open ends of the cartilage rings on the posterior aspect of the trachea. This structure plays an important role in regulating airway diameter and allowing esophageal expansion during swallowing. Identify the structure indicated by the arrow.



- a) Trachealis
- b) Hyaline cartilage
- c) Circular muscle layer of esophagus



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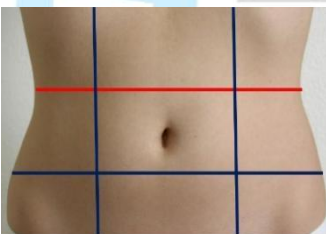
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d) Longitudinal muscle layer of esophagus

Q14. A 22-year-old man has enlarged yellow-orange tonsils, peripheral neuropathy, hepatosplenomegaly, and very low HDL-c) LDL-C and triglycerides are mildly abnormal. Biopsy of reticuloendothelial tissue shows cholesterol ester accumulation. Which transporter is defective?

- a) NPC1
- b) LDL receptor
- c) ABCA1
- d) MTP

Q15. A 28-year-old medical student is revising surface anatomy for clinical examinations. During a demonstration, a horizontal plane (marked in red) is shown passing midway between the suprasternal notch and the pubic symphysis. The instructor explains that this plane is clinically important as it passes through several key abdominal structures such as the pylorus of the stomach, neck of pancreas, and origin of the superior mesenteric artery. The student is asked to identify the vertebral level corresponding to this plane. What is the vertebral level of this plane?



- a) At junction of T12 & L1
- b) Lower border of L1
- c) Upper border of L2
- d) Upper border of L3

Q16. A 17-year-old boy develops migratory polyarthritis, fever, and a new murmur 3 weeks after untreated pharyngitis. Labs show elevated anti-streptolysin O titers. Years later, he develops exertional dyspnea, hemoptysis, atrial fibrillation, and a low-pitched diastolic rumble with opening snap. Pathology shows commissural fusion, thickened valve leaflets, and shortened chordae tendineae.

Which immune mechanism initiated the original disease?

- a) Direct bacterial invasion of valve tissue



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- b) Type II hypersensitivity due to molecular mimicry between streptococcal antigens and host tissues
- c) Type III immune complex deposition in alveolar basement membranes
- d) IgE-mediated mast cell degranulation against bacterial toxins

Q17. A 58-year-old man with familial hypercholesterolemia is treated with a drug that reduces intestinal cholesterol absorption by inhibiting a transporter at the brush border of enterocytes. LDL-C decreases further when the drug is added to a statin.

Which transporter is inhibited?

- a) NPC1L1
- b) ABCA1
- c) GLUT5
- d) SGLT1

Q18. A 43-year-old man with a history of injection drug use develops fever, pleuritic chest pain, hemoptysis, and multiple peripheral nodular lung lesions with cavitation on CT. Blood cultures grow *Staphylococcus aureus*. Echocardiography shows a large vegetation on the tricuspid valve. Which complication is most likely?

- a) Septic pulmonary emboli
- b) Systemic emboli to spleen and kidney from the right heart
- c) Coronary artery emboli causing left ventricular infarction as the most common event
- d) Sterile vegetations along the line of valve closure due to autoimmune disease

Q19. A 59-year-old man undergoes colonoscopy for occult blood-positive stool and iron deficiency anemia. A large exophytic mass is found in the ascending colon. Histology shows invasive adenocarcinoma. Molecular testing reveals chromosomal instability with APC mutation followed by KRAS activation and later p53 loss. The physician explains that this sequence differs from the microsatellite instability pathway.

Which precursor lesion most commonly gives rise to carcinoma through this pathway?

- a) Hyperplastic polyp of rectosigmoid colon with no dysplasia
- b) Conventional adenomatous polyp with dysplastic epithelium
- c) Inflammatory pseudopolyp from ulcerative colitis alone
- d) Hamartomatous juvenile polyp without dysplasia



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Q20. A 58-year-old woman develops sudden pleuritic chest pain, dyspnea, tachycardia, and mild hemoptysis 10 days after hip replacement surgery. Oxygen saturation is low. CT pulmonary angiography shows occlusion of a segmental branch of the pulmonary artery supplying the lower lobe. The affected lung region shows a peripheral, wedge-shaped, hemorrhagic lesion extending to the pleural surface. The physician explains that infarction occurred despite the lung having dual blood supply.

Which factor most strongly predisposed this patient to pulmonary infarction?

- a) High oxygen content in alveolar air preventing ischemic injury
- b) Bronchial arterial collateral supply being sufficient in all embolic events
- c) Preexisting pulmonary venous congestion or impaired bronchial circulation
- d) Absence of pleural involvement in embolic disease

Q21. A 49-year-old man has progressive exertional dyspnea and dry cough. He previously worked in ship insulation. Pulmonary function testing shows reduced total lung capacity, reduced compliance, and preserved or increased FEV1/FVC ratio. CT shows pleural plaques along the lateral chest wall and diaphragm. Years later, he develops recurrent hemorrhagic pleural effusions. Which malignancy is most strongly associated with this exposure?

- a) Small cell carcinoma
- b) Mesothelioma
- c) Pulmonary carcinoid tumor
- d) Metastatic renal cell carcinoma

Q22. A 64-year-old man presents with multiple hepatic lesions and weight loss. Biopsy shows metastatic adenocarcinoma. Immunohistochemistry reveals:

- CK7 negative
- CK20 positive
- CDX2 strongly positive

However, a second lesion shows weak CK7 positivity along with CK20 positivity, raising concern about alternative primaries.

Which of the following best explains this discrepancy?

- a) Tumor heterogeneity causing variable cytokeratin expression
- b) CK20 positivity is exclusive to colorectal origin
- c) CK7 expression excludes gastrointestinal primary tumors



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- d) CDX2 expression is specific for pancreatic adenocarcinoma

Q23. 61-year-old smoker develops central lung mass, weight loss, proximal muscle weakness, hypertension, hyperglycemia, and hypokalemic metabolic alkalosis. Biopsy reveals sheets of small blue cells with nuclear molding, scant cytoplasm, high mitotic activity, and necrosis. Immunohistochemistry is positive for chromogranin and synaptophysin.

Which additional paraneoplastic marker or hormone is most plausibly responsible for his metabolic findings?

- a) Ectopic ACTH
- b) Ectopic erythropoietin
- c) Parathyroid hormone-related peptide
- d) Calcitonin

Q24. A 66-year-old woman has metastatic adenocarcinoma in the liver. Imaging shows multiple hepatic lesions but no obvious primary tumor. Immunohistochemistry shows CK20 positive, CDX2 positive, and CK7 negative tumor cells.

Which primary site is most consistent with this immunophenotype?

- a) Colorectal adenocarcinoma
- b) Lung adenocarcinoma
- c) Breast carcinoma
- d) Ovarian serous carcinoma

Q25. A 42-year-old woman has a thyroid nodule. Fine-needle aspiration shows nests of polygonal cells with amyloid-like stromal deposits. Serum calcitonin is elevated. Congo red staining of the stromal material shows apple-green birefringence. Genetic testing reveals a RET mutation.

Which diagnosis is most likely?

- a) Papillary thyroid carcinoma
- b) Follicular thyroid carcinoma
- c) Medullary thyroid carcinoma
- d) Anaplastic thyroid carcinoma

Q26. Which of the following statements regarding this drug class is INCORRECT?



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- a) It reduces angiotensin II-mediated efferent arteriolar constriction and may decrease GFR in renal artery stenosis
- b) It may cause hyperkalemia by reducing aldosterone-mediated potassium secretion in the cortical collecting duct
- c) It is beneficial in diabetic nephropathy because it reduces intraglomerular hypertension and proteinuria
- d) It commonly causes reflex tachycardia because it directly relaxes arteriolar smooth muscle through nitric oxide release

Q27. A 67-year-old chronic smoker has progressive dyspnea, chronic productive cough, cyanosis, peripheral edema, elevated JVP, hepatomegaly, and loud P2. Arterial blood gas shows chronic hypoxemia and hypercapnia. Chest imaging shows increased bronchovascular markings rather than marked hyperinflation. Autopsy reveals enlarged mucus glands, goblet cell hyperplasia, and a Reid index greater than 50%.

Which pathological process best explains his pulmonary hypertension?

- a) Diffuse alveolar wall destruction causing loss of elastic recoil only
- b) Chronic hypoxic pulmonary vasoconstriction with vascular remodeling
- c) Left ventricular concentric hypertrophy causing pulmonary venous hypertension only
- d) Acute fibrinous pericarditis causing impaired right ventricular filling

Q28. Which of the following statements is INCORRECT?

- a) Beta-1 blockade decreases heart rate, contractility, and renin release
- b) Nonselective beta-blockers may worsen bronchospasm by blocking beta-2 receptors in bronchial smooth muscle
- c) Chronic beta-blocker therapy improves survival in selected patients with heart failure with reduced ejection fraction
- d) Beta-blockers should generally be initiated at high dose in acute decompensated systolic heart failure to immediately increase cardiac output

Q29. A 36-year-old woman presents with epigastric pain, early satiety, and weight loss. Upper GI endoscopy reveals a chronic gastric ulcer located along the lesser curvature near the pyloric region. Despite medical management, she develops hematemesis. Imaging suggests erosion of a major artery supplying that region. During surgery, the bleeding vessel is identified along the lesser curvature close to the pylorus.

Which artery is most likely involved in this case?



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- a) Left gastric
- b) Left gastroepiploic
- c) Right gastric
- d) Right gastroepiploic

Q30. A 71-year-old man develops progressive exertional dyspnea, peripheral edema, elevated JVP, hepatomegaly, and preserved ejection fraction. Echocardiography shows thickened ventricular walls with small ventricular cavities. Endomyocardial biopsy shows extracellular amorphous eosinophilic deposits that stain with Congo red and exhibit apple-green birefringence under polarized light.

Which hemodynamic abnormality best explains his symptoms?

- a) Impaired systolic contraction due to dilated ventricular chambers
- b) Impaired diastolic filling due to stiff noncompliant ventricles
- c) Dynamic left ventricular outflow obstruction due to asymmetric septal hypertrophy
- d) Acute valve destruction causing severe regurgitation

Q31. Which statement is INCORRECT?

- a) Aspirin irreversibly inhibits platelet COX-1, decreasing thromboxane A₂ synthesis
- b) Clopidogrel irreversibly inhibits the platelet P2Y₁₂ ADP receptor
- c) Abciximab inhibits GP IIb/IIIa and prevents fibrinogen-mediated platelet aggregation
- d) Platelets can restore COX activity rapidly by synthesizing new enzyme within hours after aspirin exposure

Q32. A 9-month-old infant develops failure to thrive, hepatosplenomegaly, progressive jaundice, ascites, and signs of portal hypertension. Liver biopsy shows cirrhosis and abnormal glycogen with long outer chains and very few branch points. The stored polysaccharide resembles amylopectin and provokes liver injury.

Which enzyme is deficient?

- a) Branching enzyme
- b) Debranching enzyme
- c) Glucose-6-phosphatase
- d) Muscle phosphorylase



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Q33. Which statement is INCORRECT?

- a) Methimazole inhibits thyroid peroxidase, decreasing organification and coupling reactions
- b) Propylthiouracil inhibits thyroid peroxidase and also decreases peripheral conversion of T4 to T3
- c) Agranulocytosis is a serious adverse effect of thioamides and may present with fever and sore throat
- d) Methimazole is generally preferred in the first trimester of pregnancy because it has no teratogenic risk

Q34. A 62-year-old man dies 4 days after severe crushing chest pain radiating to the left arm. He initially had ST elevations in leads II, III, and aVF. Autopsy shows a pale infarcted area in the posterior left ventricle. Microscopy reveals extensive coagulative necrosis with dense neutrophilic infiltration and early macrophage recruitment, but no mature collagen deposition.

Which complication is most likely during the next several days if he had survived?

- a) Ventricular free wall rupture due to macrophage-mediated weakening
- b) Dense fibrous scar causing chronic aneurysm immediately
- c) Constrictive pericarditis from calcified pericardium
- d) Dilated cardiomyopathy from viral myocarditis

Q35. Which statement is INCORRECT?

- a) Methotrexate inhibits dihydrofolate reductase, reducing tetrahydrofolate regeneration
- b) Leucovorin can rescue normal cells from high-dose methotrexate toxicity because it bypasses dihydrofolate reductase
- c) 5-fluorouracil is converted to FdUMP, which inhibits thymidylate synthase
- d) 5-fluorouracil directly inhibits dihydrofolate reductase, and leucovorin reverses its antitumor activity

Q36. A toddler has fasting hypoglycemia, postprandial hyperglycemia, hepatomegaly, growth failure, glucosuria, phosphaturia, aminoaciduria, and rickets. Genetic testing shows defective facilitative glucose transport in liver, pancreatic beta cells, intestine, and renal tubules. Which transporter is defective?

- a) GLUT1
- b) GLUT2
- c) GLUT4
- d) SGLT1



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Q37. A 28-year-old man with known generalized epilepsy is brought to the emergency department after a witnessed tonic-clonic seizure lasting 9 minutes. He is unresponsive, cyanotic, and has persistent rhythmic limb jerking. His airway is supported, oxygen is given, IV access is obtained, and bedside glucose is normal. The team prepares an immediate anticonvulsant while also planning a longer-acting agent to prevent recurrence after seizure termination.

Regarding the drug of choice for immediate termination of this episode, all of the following are true EXCEPT:

- a) IV lorazepam is preferred acutely because it rapidly enhances GABA-A receptor-mediated chloride influx
- b) Diazepam has rapid CNS entry but redistributes quickly into peripheral tissues, making recurrence more likely if no longer-acting drug is added
- c) Fosphenytoin or levetiracetam may be added after benzodiazepine therapy to prevent recurrent seizures
- d) Oral phenytoin is the preferred first intervention because it rapidly terminates convulsive status epilepticus without respiratory depression

Q38. A child with glucose-6-phosphatase deficiency develops gout-like symptoms as he ages. Labs show elevated uric acid. The physician explains that uric acid is elevated due to both increased production and decreased renal excretion. Which mechanism contributes to decreased uric acid excretion?

- a) Lactate competes with urate for renal tubular secretion
- b) Excess ammonia blocks urate filtration at the glomerulus
- c) Ketone bodies irreversibly bind uric acid in plasma
- d) Insulin directly converts uric acid to allantoin

Q39. A 17-year-old girl develops generalized urticaria, throat tightness, wheezing, hypotension, vomiting, and syncope minutes after eating peanuts. On examination, she has stridor, diffuse bronchospasm, weak pulses, and blood pressure of 70/40 mmHg. Oxygen and IV fluids are started. The physician administers the drug of choice intramuscularly into the lateral thigh.

Regarding this drug, all of the following are true EXCEPT:

- a) It stimulates alpha-1 receptors, reversing vasodilation and mucosal edema
- b) It stimulates beta-2 receptors, producing bronchodilation and reducing mediator release from mast cells
- c) It stimulates beta-1 receptors, increasing heart rate and contractility during cardiovascular collapse



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- d) It should be delayed until antihistamines and corticosteroids have had adequate time to reverse airway edema

Q40. A newborn has elevated phenylalanine on screening. Despite dietary phenylalanine restriction, he develops severe neurologic symptoms, seizures, temperature instability, and abnormal movements. CSF studies show low dopamine and serotonin metabolites. Dihydrobiopterin reductase activity is deficient. Which cofactor-related abnormality best explains this presentation?

- a) Impaired regeneration of tetrahydrobiopterin
- b) Impaired conversion of methylmalonyl-CoA to succinyl-CoA
- c) Impaired transport of cystine from lysosomes
- d) Impaired conversion of propionyl-CoA to methylmalonyl-CoA

Q41. A 61-year-old man with long-standing hypertension presents with sudden tearing chest pain radiating to the back. Blood pressure is 220/120 mmHg in the right arm and 180/100 mmHg in the left arm. CT angiography shows an ascending aortic dissection. The ICU team plans rapid reduction of shear stress by first lowering heart rate and contractility before adding vasodilator therapy if needed.

Regarding the preferred initial pharmacologic strategy, all of the following are true EXCEPT:

- a) IV beta-blockade with esmolol or labetalol reduces heart rate, contractility, and aortic wall shear stress
- b) Sodium nitroprusside may be added after adequate beta-blockade if blood pressure remains elevated
- c) Starting a potent arterial vasodilator before beta-blockade may worsen reflex tachycardia and increase shear stress
- d) Immediate oral ACE inhibitor monotherapy is preferred because gradual onset avoids cerebral hypoperfusion

Q42. A 34-year-old woman with untreated Graves disease presents with fever, severe agitation, diarrhea, tremor, atrial fibrillation with rapid ventricular response, and heart failure. Labs show suppressed TSH and markedly elevated free T4 and T3. She is diagnosed with thyroid storm. The team begins treatment with a beta-blocker, antithyroid drug, iodine, glucocorticoid, and supportive cooling.

Regarding the drug-of-choice strategy in thyroid storm, all of the following are true EXCEPT:



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- a) Propylthiouracil is often preferred initially because it inhibits thyroid peroxidase and peripheral conversion of T₄ to T₃
- b) Iodine solution should be given before thioamide therapy to immediately block new thyroid hormone synthesis
- c) Beta-blockers reduce adrenergic symptoms, and propranolol also decreases peripheral T₄-to-T₃ conversion at high doses
- d) Glucocorticoids reduce peripheral T₄-to-T₃ conversion and treat possible relative adrenal insufficiency

Q43. A 69-year-old man with chronic kidney disease stage 4, heart failure, and a recent bleeding gastric ulcer develops abrupt severe pain, swelling, and erythema of the first metatarsophalangeal joint. Arthrocentesis reveals negatively birefringent needle-shaped crystals. He is already taking allopurinol chronically. The clinician avoids NSAIDs because of renal disease and ulcer history.

Regarding pharmacologic management, all of the following are true EXCEPT:

- a) Intra-articular or systemic corticosteroids are reasonable options for acute gout when NSAIDs and colchicine are problematic
- b) Allopurinol should not be newly initiated during an acute flare solely for immediate pain control
- c) Colchicine can cause gastrointestinal toxicity and requires caution or dose adjustment in renal impairment
- d) High-dose aspirin is the preferred acute treatment because it rapidly lowers urate and is safe in active peptic ulcer disease

Q44. A 68-year-old woman is brought to the emergency department with sudden-onset nausea, vomiting, vertigo, and gait unsteadiness. She is unable to swallow properly and has marked hoarseness of voice. Neurological examination reveals ataxia and impaired pain and temperature sensation over one side of the body. The constellation of findings suggests involvement of the lateral medulla, with affection of the vestibular nuclei, nucleus ambiguus, spinothalamic tract, and inferior cerebellar connections.

Occlusion of which artery most likely explains this presentation?

- a) Posterior inferior cerebellar artery
- b) Internal carotid artery
- c) Anterior inferior cerebellar artery
- d) Basilar artery



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Q45. A 19-year-old man undergoing surgery after succinylcholine and inhaled sevoflurane develops rapidly rising end-tidal CO₂, generalized muscle rigidity, hyperthermia, tachycardia, metabolic acidosis, and hyperkalemia. The anesthesiologist stops triggering agents and administers the drug of choice.

Regarding this drug and syndrome, all of the following are true EXCEPT:

- a) Dantrolene treats malignant hyperthermia by inhibiting calcium release from the sarcoplasmic reticulum through ryanodine receptor effects
- b) Succinylcholine and volatile anesthetics are classic triggers in susceptible patients
- c) The condition involves uncontrolled skeletal muscle contraction, increased ATP consumption, heat generation, and rhabdomyolysis
- d) Neostigmine is the drug of choice because it reverses succinylcholine-induced depolarization at the neuromuscular junction

Q46. A 42-year-old farmer is found confused, sweating, wheezing, vomiting, and salivating excessively after pesticide exposure. Examination shows miosis, bradycardia, bronchorrhea, fasciculations, and muscle weakness. He is treated urgently with airway support and antidotes.

Regarding drug-of-choice therapy, all of the following are true EXCEPT:

- a) Atropine competitively antagonizes muscarinic acetylcholine receptors and improves bronchorrhea, bronchospasm, bradycardia, and secretions
- b) Pralidoxime can regenerate acetylcholinesterase if given before aging of the enzyme-inhibitor complex
- c) Pralidoxime is especially important for nicotinic manifestations such as fasciculations and skeletal muscle weakness
- d) Atropine reverses both muscarinic and nicotinic manifestations by reactivating acetylcholinesterase

Q47. A newborn has intermittent encephalopathy and elevated branched-chain amino acids. Genetic testing reveals a residual activity variant of the branched-chain alpha-ketoacid dehydrogenase complex. The physician recommends dietary leucine restriction and a vitamin trial. Which vitamin may improve enzyme activity in some variants?

- a) Riboflavin
- b) Niacin
- c) Thiamine
- d) Vitamin C



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Q48. An 85-year-old woman presents with progressive difficulty in chewing and unintentional weight loss. On examination, there is weakness in jaw movements, particularly while opening the mouth. Neurological evaluation suggests involvement of muscles responsible for mandibular depression. The clinician recalls that most muscles of mastication elevate the mandible, but one specific muscle plays a key role in opening the jaw by protruding and depressing the mandible.

Which of the following muscles acts as a depressor of the mandible?

- a) Buccinator muscle
- b) Lateral pterygoid muscle
- c) Medial pterygoid muscle
- d) Posterior fibers of temporalis muscle

Q49. A 31-year-old man is found unconscious with pinpoint pupils, respiratory rate of 5/min, cyanosis, and needle marks. EMS provides ventilation and gives IV naloxone. He wakes briefly but becomes somnolent again 45 minutes later. Toxicology later suggests a long-acting opioid exposure.

Regarding the drug of choice and clinical pharmacology, all of the following are true EXCEPT:

- a) Naloxone is a competitive opioid receptor antagonist used for acute opioid-induced respiratory depression
- b) Recurrent respiratory depression can occur because naloxone has a shorter duration of action than many opioids
- c) Airway and ventilatory support remain essential even when naloxone is administered
- d) Naloxone should be avoided in all opioid overdoses because it invariably causes fatal withdrawal

Q50. A 62-year-old patient with carcinoma of the oral cavity is planned for neck dissection. During preoperative discussion, the surgeon quizzes a junior resident about lymphatic drainage patterns of the face and oral cavity. He emphasizes that understanding the drainage pathways is crucial for predicting the spread of malignancy. The resident is asked specifically about the submental group of lymph nodes, which lie beneath the chin and are often involved in early metastasis from midline structures.

Which of the following areas drains primarily into the submental lymph nodes?

- a) Upper lip
- b) Parotid gland
- c) Upper incisors
- d) Tip of the tongue



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Q51. A 27-year-old pregnant woman at 35 weeks has severe headache, visual symptoms, right upper quadrant pain, blood pressure of 170/112 mmHg, thrombocytopenia, elevated AST/ALT, proteinuria, and hyperreflexia. While being evaluated, she has a generalized tonic-clonic seizure. The obstetric team administers the drug of choice for seizure treatment and prevention in eclampsia while also controlling severe hypertension and planning delivery.

Regarding this drug, all of the following are true EXCEPT:

- a) Magnesium sulfate is the drug of choice for seizure prophylaxis and treatment in eclampsia
- b) Toxicity may cause loss of deep tendon reflexes, respiratory depression, and cardiac conduction abnormalities
- c) Calcium gluconate can be used as an antidote for severe magnesium toxicity
- d) Phenytoin is preferred over magnesium sulfate because it more effectively prevents recurrent eclamptic seizures

Q52. A 32-year-old man with bipolar disorder presents with 1 week of decreased need for sleep, grandiosity, pressured speech, risky spending, and agitation. He has no psychosis or substance intoxication. His psychiatrist considers long-term mood stabilization. Baseline labs include creatinine, TSH, and electrolytes. The patient is counseled about narrow therapeutic index, tremor, polyuria, hypothyroidism, and toxicity risk with dehydration or NSAIDs.

Regarding this drug of choice for classic maintenance therapy, all of the following are true EXCEPT:

- a. Lithium can cause nephrogenic diabetes insipidus by impairing collecting duct response to ADH
- b. Lithium toxicity risk increases with dehydration, renal impairment, thiazide diuretics, ACE inhibitors, and NSAIDs
- c. Lithium can cause hypothyroidism and requires thyroid monitoring
- d. Lithium is metabolized extensively by hepatic CYP450 enzymes, so dose adjustment is mainly based on liver function

Q53. A 7-month-old infant has failure to thrive, cabbage-like body odor, vomiting, jaundice, hepatomegaly, coagulopathy, renal Fanconi syndrome, and episodes of painful peripheral neuropathy. Labs show elevated tyrosine and succinylacetone.

Which enzyme is deficient?

- a) Tyrosine aminotransferase
- b) Homogentisate oxidase
- c) Fumarylacetoacetate hydrolase



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d) Phenylalanine hydroxylase

Q54. A patient has contralateral hemiparesis, contralateral loss of vibration/proprioception, and ipsilateral tongue deviation. Which artery is occluded?

- a) Posterior inferior cerebellar artery
- b) Anterior spinal artery
- c) Superior cerebellar artery
- d) Middle cerebral artery

Q55. A man using crutches develops weakness of wrist extension, finger extension, arm extension, and sensory loss over the posterior arm, forearm, and dorsolateral hand. Which lesion best explains all findings?

- a) Radial nerve in spiral groove
- b) Posterior cord of brachial plexus
- c) Axillary nerve alone
- d) Musculocutaneous nerve

Q56. A child with tyrosinemia type I is treated with nitisinone and dietary restriction of tyrosine and phenylalanine. The drug prevents formation of toxic downstream metabolites such as fumarylacetoacetate and succinylacetone.

Which enzyme does nitisinone inhibit?

- a) Phenylalanine hydroxylase
- b) Tyrosine aminotransferase
- c) 4-hydroxyphenylpyruvate dioxygenase
- d) Homogentisate oxidase

Q57. A 7-year-old child develops intermittent ataxia, mood changes, diarrhea, and a photosensitive pellagra-like rash after a period of poor nutrition. Urine amino acid analysis shows increased neutral amino acids, especially tryptophan.

Which vitamin deficiency explains the rash and neuropsychiatric symptoms?

- a) Vitamin B1
- b) Vitamin B2
- c) Vitamin B3



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d) Vitamin B6

Q58. A 69-year-old man develops asymmetric resting tremor, bradykinesia, rigidity, and reduced arm swing. Over the next several years, he develops postural instability and cognitive decline. Autopsy shows depigmentation of the substantia nigra pars compacta. Microscopy shows intracytoplasmic eosinophilic inclusions in surviving neurons.

Which protein is the major component of these inclusions?

- a) Beta-amyloid
- b) Hyperphosphorylated tau
- c) Alpha-synuclein
- d) Huntingtin

Q59. A patient initially has vague periumbilical pain, later localizing to the right lower quadrant.

Which embryologic explanation best accounts for initial pain location?

- a) Appendix is foregut-derived
- b) Appendix is midgut-derived and refers pain to T10 dermatome
- c) Appendix is hindgut-derived and refers pain to S2–S4
- d) Appendix is supplied only by somatic nerves

Q60. A 74-year-old man with chronic alcohol use is brought to clinic by his daughter because of worsening confusion, headache, gait instability, and mild right-sided weakness over the past 3 weeks. He fell from bed 1 month ago but did not seek care because he felt well afterward. CT scan shows a crescent-shaped extra-axial collection over the left cerebral convexity that crosses suture lines but does not cross dural reflections. The collection contains blood products of different ages.

Which vessel is most likely responsible?

- a) Middle meningeal artery
- b) Bridging cortical veins
- c) Anterior communicating artery
- d) Lenticulostriate artery

Q61. A 31-year-old woman has recurrent episodes of neurologic dysfunction separated in time and space. She previously had painful monocular visual loss that improved over several weeks.



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Now she presents with limb paresthesias, internuclear ophthalmoplegia, and urinary urgency. MRI shows multiple periventricular plaques oriented perpendicular to the lateral ventricles. CSF analysis reveals oligoclonal IgG bands.

Which pathological process best explains her disease?

- a) Autoimmune demyelination of CNS white matter with relative axonal preservation
- b) Viral destruction of oligodendrocytes by JC virus in immunosuppression
- c) Peripheral nerve segmental demyelination by macrophages
- d) Ischemic necrosis of anterior horn cells

Q62. A 78-year-old woman gradually develops impaired recent memory, word-finding difficulty, visuospatial impairment, and later personality changes. There is no stepwise decline. Brain biopsy in a research protocol shows cortical atrophy, extracellular neuritic plaques, and intracellular neurofibrillary tangles. The plaques are derived from abnormal cleavage of a transmembrane precursor protein.

Which molecular event is most directly involved in plaque formation?

- a) Accumulation of alpha-synuclein in cytoplasmic inclusions
- b) Abnormal beta-amyloid deposition derived from amyloid precursor protein
- c) Expansion of CAG repeats in the huntingtin gene
- d) Accumulation of misfolded prion protein causing spongiform change

Q63. A man sustains straddle injury with rupture of spongy urethra. Urine extravasates into superficial perineal pouch and anterior abdominal wall.

Which fascia limits spread into the thigh?

- a) Scarpa fascia attachment to fascia lata
- b) Colles fascia opening into ischioanal fossa
- c) Buck fascia attachment to pubic symphysis
- d) Camper fascia attachment to inguinal ligament

Q64. A 30-year-old man is brought to the emergency department following a high-speed motorcycle accident. CT scan reveals a basilar skull fracture involving the region of the foramen ovale. On neurological examination, the patient shows weakness while chewing and difficulty in clenching the jaw. There is no significant sensory loss over the face, but motor functions of



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mastication appear compromised. The clinician suspects injury to the nerve passing through the foramen ovale.

Which of the following functional deficits is most likely seen in this patient?

- a) Loss of abduction of the eye
- b) Loss of sensation over the angle of mandible
- c) Paralysis of genioglossus
- d) Paralysis of temporalis muscle

Q65. A 65-year-old patient presents with episodes of dizziness and syncope. On examination, the clinician observes the jugular venous pulse (JVP) and notes intermittently large, prominent pulsations in the neck. These waves appear irregular and are not synchronized with the arterial pulse. The physician suspects a condition where the atria contract against a closed tricuspid valve, producing exaggerated venous pulsations known as “cannon waves.” The marked wave labeled ‘1’ in the image corresponds to this phenomenon. In which clinical condition are such prominent waves most likely seen?



- a) Tricuspid stenosis
- b) Pulmonary hypertension
- c) Atrial fibrillation
- d) Complete heart block

Q66. A 24-year-old woman has chronic bloating, foul-smelling stools, weight loss, iron deficiency anemia, and a pruritic vesicular rash on the extensor surfaces of her elbows. Symptoms improve after removing wheat, rye, and barley from her diet. Small intestinal biopsy shows villous atrophy, crypt hyperplasia, and increased intraepithelial lymphocytes, most prominent in the proximal small intestine.

Which immune mechanism is central to this disease?

- a) IgE-mediated mast cell degranulation against dietary gluten
- b) T-cell mediated sensitivity to deamidated gliadin
- c) Autoimmune destruction of gastric parietal cells



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d) Infection-induced blunting of villi throughout the small bowel responsive only to antibiotics

Q67. A pelvic tumor compresses nerves along the lateral pelvic wall and sacral plexus. The patient has difficulty walking, sensory loss in the leg, and pelvic pain. Which statement is INCORRECT?

- a) Obturator nerve injury causes weakness of thigh adduction
- b) Superior gluteal nerve injury causes Trendelenburg gait
- c) Inferior gluteal nerve injury weakens hip extension from a flexed position
- d) Femoral nerve injury causes loss of plantar flexion and Achilles reflex

Q68. A 78-year-old woman gradually develops impaired recent memory, word-finding difficulty, visuospatial impairment, and later personality changes. There is no stepwise decline. Brain biopsy in a research protocol shows cortical atrophy, extracellular neuritic plaques, and intracellular neurofibrillary tangles. The plaques are derived from abnormal cleavage of a transmembrane precursor protein.

Which molecular event is most directly involved in plaque formation?

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- c) Expansion of CAG repeats in the huntingtin gene
- d) Accumulation of misfolded prion protein causing spongiform change

Q69. A 42-year-old man with chronic cough, fever, night sweats, weight loss, and apical cavitory lung lesions provides three early-morning sputum samples. One smear shows slender red rods after carbol fuchsin staining and acid-alcohol decolorization. The clinician sends culture and nucleic acid amplification testing because smear positivity alone cannot fully determine species identity or drug susceptibility. The laboratory warns that growth may take weeks due to the organism's lipid-rich cell wall and slow replication.

Regarding the staining and culture of the likely organism, all of the following are true EXCEPT:

- a) Ziehl-Neelsen stain demonstrates acid-fast bacilli due to mycolic acid-rich cell walls
- b) Auramine-rhodamine fluorescent staining can be used as a sensitive screening method for acid-fast bacilli
- c) Lowenstein-Jensen medium may be used for culture, but growth is slow and may require several weeks
- d) Acid-fast smear positivity reliably distinguishes *Mycobacterium tuberculosis* from all nontuberculous mycobacteria without further testing



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Q70. A 30-year-old man with known HIV infection, on antiretroviral therapy for the past 3 years, presents with gradually worsening pain in the right hip for the last 2 months. The pain is insidious in onset, increases on weight-bearing, and has progressively limited his mobility. There is no history of trauma or fever. On examination, the hip joint is held in a position of flexion, abduction, and external rotation, with painful restriction of movements. There are no systemic signs of infection.

What is the most likely diagnosis?



- a) TB Hip
- b) Avascular necrosis
- c) Transient synovitis
- d) Septic arthritis

Q71. A 55-year-old patient is admitted to the ICU with acute respiratory distress and requires supplemental oxygen therapy. The oxygen is delivered through a central pipeline system connected to wall-mounted outlets. The attending anesthesiologist explains that medical gases are supplied at a standardized pressure to ensure safe and consistent delivery across different hospital units. He emphasizes the importance of knowing this pressure for proper functioning of flowmeters and ventilators.

What is the standard pressure at which oxygen is supplied through the central pipeline system?

- a) 30 pounds per square inch (psi)
- b) 50 psi
- c) 100 psi
- d) 60 psi



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Q72. A 24-year-old woman presents to the emergency department with high-grade fever, vomiting, and diffuse erythematous rash. She is hypotensive and appears toxic. On further history, she reports prolonged use of a high-absorbency tampon during menstruation. Examination reveals desquamation of the palms and soles after a few days.

Laboratory investigations show multisystem involvement, including renal dysfunction and elevated liver enzymes. Blood cultures may be negative, but toxin-mediated effects are suspected. The causative organism produces a superantigen toxin that leads to massive cytokine release and systemic manifestations.

Which of the following organisms is most likely responsible?

- a) *Staphylococcus aureus*
- b) *Streptococcus viridans*
- c) *Streptococcus pyogenes*
- d) *Candida albicans*

Q73. The microbiology laboratory notes that although cytopathic effects may be seen in culture, diagnosis is often made by more rapid techniques, especially in immunocompromised hosts where viral load correlates with disease severity.

Regarding the organism and its diagnostic evaluation, all of the following are true EXCEPT:

- a) The described intranuclear inclusion morphology reflects viral-mediated nuclear enlargement with chromatin margination, often termed an “owl’s eye” appearance
- b) Quantitative PCR assays are useful not only for diagnosis but also for monitoring treatment response and predicting disease progression in immunocompromised patients
- c) Shell vial culture techniques using monoclonal antibodies can accelerate detection compared to conventional viral culture
- d) Cytopathic changes observed on Tzanck smear reliably distinguish this virus from other herpesviruses due to its unique multinucleated giant cell formation

Q74. A 5-year-old child is brought to the emergency department with high-grade fever, vomiting, and neck stiffness. Suspecting acute bacterial meningitis, a lumbar puncture is performed, and cerebrospinal fluid (CSF) is sent for urgent analysis. The clinician requires a rapid bedside test to detect microbial antigens within minutes to initiate early targeted therapy.

The laboratory technician uses a method in which inert particles are coated with specific antibodies, and visible clumping occurs when corresponding microbial antigens are present in the sample. The test



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provides results in less than 5 minutes and is commonly used for detecting pathogens like *Streptococcus pneumoniae* and *Haemophilus influenzae* in CSF.

Which of the following tests is being described?

- a) Coagglutination (COA)
- b) Counterimmunoelectrophoresis (CIE)
- c) Enzyme-linked immunosorbent assay (ELISA)
- d) Latex agglutination

Q75. A 70-year-old patient receives empiric antibiotics prior to sputum collection for suspected pneumonia. Later Gram stain shows abundant neutrophils but few organisms. Blood culture grows alpha-hemolytic, optochin-sensitive diplococci.

Which statement is INCORRECT?

- a) Prior antibiotic exposure can reduce bacterial yield in both smear and culture
- b) Optochin sensitivity helps distinguish *Streptococcus pneumoniae* from viridans streptococci
- c) Capsule contributes to virulence and interferes with phagocytosis
- d) Absence of organisms on Gram stain excludes pneumococcal infection

Q76. All of the following statements are true EXCEPT:

- a) Pott puffy tumor refers to frontal bone osteomyelitis with subperiosteal abscess, often related to frontal sinusitis
- b) Intracranial complications of sinusitis may include epidural abscess, subdural empyema, brain abscess, and meningitis
- c) Cavernous sinus thrombosis can involve cranial nerves III, IV, V1, V2, and VI, with CN VI often affected early due to its central location
- d) Cavernous sinus thrombosis is excluded if the patient has ocular motor palsies because cranial nerves do not traverse or border the cavernous sinus

Q77. A patient presents with urethritis. Gram stain shows intracellular gram-negative diplococci. Culture is performed using selective enriched media containing antibiotics to suppress commensals.

Which statement is INCORRECT?

- a) Selective media enhance recovery of fastidious pathogens by suppressing competing flora
- b) Chocolate agar supports growth of organisms requiring released intracellular nutrients
- c) Oxidase positivity supports identification of *Neisseria* species



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d) Growth on MacConkey agar with lactose fermentation is characteristic of this organism

Q78. All of the following statements are true EXCEPT:

- a) Symptoms lasting less than 10 days with gradual improvement are more consistent with viral rhinosinusitis than uncomplicated bacterial sinusitis
- b) “Double worsening” after initial improvement supports acute bacterial rhinosinusitis
- c) Routine CT imaging is required for every uncomplicated acute rhinosinusitis episode before starting medical treatment
- d) Orbital signs, neurologic symptoms, severe systemic toxicity, or suspected complications justify urgent imaging

Q79. An immunocompromised patient has meningitis. CSF shows encapsulated yeast with halo-like appearance. Which statement is INCORRECT according to this organism ?

- a) Capsule visualization may be achieved using negative staining
- b) Antigen detection is more sensitive than microscopy
- c) Mucicarmine highlights capsular polysaccharide
- d) Germ tube formation is characteristic for this organism

Q80. All of the following statements are true EXCEPT:

- a) Allergic fungal rhinosinusitis is a noninvasive hypersensitivity reaction to fungal elements, often associated with nasal polyposis and allergic mucin
- b) Invasive fungal rhinosinusitis in diabetics or immunocompromised patients may show angioinvasion, tissue necrosis, orbital involvement, and intracranial extension
- c) Mucormycosis classically shows broad, ribbon-like, pauciseptate hyphae with right-angle branching
- d) Allergic fungal rhinosinusitis is defined by vascular invasion and must always be treated as acute fulminant rhinocerebral mucormycosis

Q81. Which of the following statements is INCORRECT?

- a) Acid-fast staining detects mycolic acid-containing cell walls irrespective of organism viability
- b) Positive acid-fast smear may persist even after effective therapy due to nonviable organisms
- c) NAAT can distinguish viable from nonviable mycobacteria more reliably than culture
- d) Culture remains the gold standard for confirming viable organisms and drug susceptibility

Q82. All of the following statements are true EXCEPT:



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- a) Inverted papilloma commonly arises from the lateral nasal wall and may cause unilateral nasal obstruction
- b) It has a tendency to recur if incompletely excised
- c) It may be associated with synchronous or metachronous squamous cell carcinoma
- d) It is a bilateral allergic inflammatory polyp with no recurrence risk and no malignant association

Q83. A fungal infection shows yeast in tissue but mold in culture. The clinician incorrectly assumes the organism is *Candida* due to yeast form. Which statement is INCORRECT compare and answer?

- a) Dimorphic fungi change morphology based on temperature
- b) Tissue morphology alone may mislead diagnosis
- c) *Candida* species show thermal dimorphism identical to *Histoplasma*
- d) Culture morphology helps confirm organism identity

Q84. All of the following statements are true EXCEPT:

- a) Isolated recurrent laryngeal nerve injury after thyroid surgery can cause unilateral vocal cord paralysis with hoarseness and aspiration risk
- b) The recurrent laryngeal nerve supplies all intrinsic muscles of the larynx except cricothyroid
- c) Palatal droop, impaired gag reflex, and shoulder weakness suggest a higher lesion involving multiple lower cranial nerves rather than isolated recurrent laryngeal nerve injury
- d) Hypoglossal nerve injury is the most common explanation for isolated paramedian vocal cord paralysis after thyroidectomy

Q85. All of the following statements are true EXCEPT:

- a) Modified Thayer-Martin medium supports recovery of pathogenic *Neisseria* while inhibiting competing flora
- b) Chocolate agar provides nutrients released from lysed red blood cells and supports growth of fastidious organisms
- c) Culture remains useful when antimicrobial susceptibility information is required
- d) *Neisseria gonorrhoeae* is best recovered as a lactose-fermenting colony on MacConkey agar

Q86. All of the following statements are true EXCEPT:

- a) Airway evaluation and early airway control may be required before abscess drainage in patients with impending obstruction



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- b) Odontogenic deep neck infections are often polymicrobial, including aerobic and anaerobic oral flora
- c) Contrast-enhanced CT helps define involved fascial spaces, abscess formation, gas, and mediastinal extension risk
- d) Needle aspiration of the tonsillar pillar alone is definitive management for all deep neck space infections regardless of airway compromise or multiloculated spread

Q87. All of the following statements are true EXCEPT:

- a) *Histoplasma capsulatum* commonly appears as small intracellular yeasts within macrophages in tissue
- b) Sabouraud dextrose agar can be used for fungal culture, though growth may be slow
- c) Dimorphic fungi may grow as mold at lower temperature and yeast at body temperature
- d) Broad-based budding yeasts with thick refractile walls are the characteristic tissue form of *Histoplasma capsulatum*

Q88. All of the following statements are true EXCEPT:

- a) Malignant otitis externa is classically associated with diabetes or immunocompromise and commonly involves *Pseudomonas aeruginosa*
- b) Cranial neuropathies may occur due to skull base osteomyelitis and indicate advanced disease
- c) Severe nocturnal otalgia with granulation tissue in the external auditory canal is a concerning clinical pattern
- d) The condition is usually a self-limited superficial fungal infection treated definitively with simple wax removal alone

Q89. All of the following statements are true EXCEPT:

- a) Metachromatic granules may be demonstrated with special stains such as Albert stain
- b) Tellurite-containing media can aid isolation of *Corynebacterium diphtheriae*
- c) Detection of toxigenicity is important because severe systemic manifestations are toxin mediated
- d) Culture positivity alone proves toxin-mediated diphtheria without need for toxin detection or clinical correlation

Q90. All of the following statements are true EXCEPT:

- a) Non-lactose fermentation on MacConkey agar helps narrow the differential among enteric gram-negative rods
- b) Lack of motility and absence of H₂S production support *Shigella* over many *Salmonella* isolates



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- c) MacConkey agar helps differentiate lactose-fermenting from non-lactose-fermenting gram-negative rods
- d) Strong urease positivity with swarming motility is characteristic of *Shigella* species

Q91. All of the following statements are true EXCEPT:

- a) Posterior canal BPPV classically produces brief, fatigable, positional vertigo with latency on Dix-Hallpike testing
- b) The Epley maneuver is designed to reposition canaliths from the posterior semicircular canal toward the utricle
- c) Persistent downbeat nystagmus with cerebellar signs suggests central positional vertigo rather than uncomplicated BPPV
- d) Central positional vertigo is reliably excluded if vertigo is triggered by head position

Q92. Which of the following statements is INCORRECT?

- a) HIV RNA PCR becomes positive before p24 antigen and antibody detection during acute infection
- b) A negative 4th-generation HIV test at 3 weeks definitively excludes early HIV infection
- c) High viral load during acute infection correlates with increased transmissibility
- d) Seroconversion occurs after initial viremia, leading to detectable antibodies

Q93. All of the following statements are true EXCEPT:

- a) Ménière disease is associated with endolymphatic hydrops
- b) Vertigo attacks typically last minutes to hours and are accompanied by auditory symptoms
- c) Hearing loss is classically fluctuating and initially affects low frequencies
- d) The usual pathology is bilateral vestibular schwannoma arising from cochlear hair cells

Q94. A 30-year-old healthcare worker presents after needle-stick exposure. Serial serology is performed.

Findings:

- HBsAg: negative
- Anti-HBs: negative
- Anti-HBc IgM: positive

Which statement is INCORRECT?



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- a) This serologic pattern is consistent with the window period
- b) Anti-HBc IgM is the most reliable marker during this phase
- c) HBsAg disappears before anti-HBs appears
- d) Anti-HBs positivity would confirm active infection

Q95. All of the following statements are true EXCEPT:

- a) Fixation of the stapes footplate at the oval window produces conductive hearing loss
- b) Carhart notch represents an apparent depression in bone conduction, classically around 2 kHz
- c) Pregnancy may worsen symptoms in some patients due to hormonal influence
- d) The primary pathology is purulent destruction of the ossicular chain by middle-ear infection

Q96. A 55-year-old man develops severe influenza during a seasonal outbreak. Viral sequencing shows point mutations in hemagglutinin gene. No reassortment is observed.

Which statement is INCORRECT?

- a) Antigenic drift involves gradual mutations in viral genes
- b) Antigenic shift involves reassortment of segmented genomes
- c) Antigenic drift is responsible for pandemics
- d) Drift allows partial immune escape

Q97. All of the following statements are true EXCEPT:

- a) Attic cholesteatoma commonly causes bone erosion due to enzymatic osteitis and pressure effects
- b) Foul-smelling scanty discharge with attic retraction suggests unsafe chronic suppurative otitis media
- c) Canal wall down mastoidectomy may be considered when disease is extensive or follow-up is unreliable
- d) Cholesteatoma is primarily a malignant epithelial tumor with early hematogenous metastasis

Q98. All of the following statements are true EXCEPT:

- a) Autoantibodies are directed against desmogleins in desmosomes, causing loss of keratinocyte adhesion
- b) The blister is intraepidermal and therefore tends to be flaccid and fragile
- c) Direct immunofluorescence classically shows linear IgG and C3 deposition along the basement membrane zone



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- d) Mucosal involvement is common because desmoglein expression patterns differ between mucosa and skin

Q99. All of the following statements are true EXCEPT:

- a) The cutaneous disease is associated with gluten-sensitive enteropathy
- b) Granular IgA deposition in dermal papillae is a classic immunofluorescence finding
- c) Dapsone may rapidly improve skin lesions, although gluten avoidance is important for intestinal disease control
- d) Linear IgG deposition along the basement membrane is the diagnostic immunofluorescence pattern

Q100. All of the following statements are true EXCEPT:

- a) Koebner phenomenon describes development of lesions at sites of trauma
- b) Munro microabscesses refer to neutrophils in the stratum corneum
- c) Psoriasis may be associated with seronegative inflammatory arthritis and nail changes
- d) The disease is primarily caused by IgE-mediated mast cell degranulation against environmental allergens

Q101. All of the following statements are true EXCEPT:

- a) Erythema multiforme is commonly associated with herpes simplex infection
- b) Classic target lesions are typical and often acral in distribution
- c) Extensive mucosal necrosis with large sheets of epidermal detachment is required for uncomplicated erythema multiforme minor
- d) The condition is immune mediated and differs clinically from toxic epidermal necrolysis

Q102. All of the following statements are true EXCEPT:

- a) Breslow thickness is one of the most important prognostic factors in localized melanoma
- b) Ulceration worsens prognosis and affects staging
- c) Sentinel lymph node biopsy may be considered depending on tumor thickness and other high-risk features
- d) ABCDE criteria are sufficient to determine prognosis and replace histologic staging

Q103. All of the following statements are true EXCEPT:

- a) Basal cell carcinoma commonly shows peripheral palisading and stromal retraction on histology
- b) Basal cell carcinoma is locally invasive but rarely metastasizes



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- c) Squamous cell carcinoma may show keratin pearls and has greater metastatic potential than basal cell carcinoma, especially on lip, ear, or immunosuppressed patients
- d) Basal cell carcinoma characteristically arises from melanocytes and strongly expresses HMB-45 as its defining marker

Q104. All of the following statements are true EXCEPT:

- a) Palpable purpura suggests inflammation and damage of small dermal blood vessels
- b) Leukocytoclastic vasculitis shows neutrophilic debris and fibrinoid necrosis of vessel walls
- c) Drug exposure, infection, autoimmune disease, and IgA vasculitis are possible causes
- d) Nonpalpable petechiae due solely to thrombocytopenia and leukocytoclastic vasculitis are histologically identical processes

Q105. All of the following statements are true EXCEPT:

- a) Dermatophytes infect keratinized tissues such as stratum corneum, hair, and nails
- b) KOH dissolves keratin and allows visualization of fungal hyphae
- c) Topical corticosteroid use can modify appearance and worsen dermatophyte infection, producing tinea incognito
- d) Dermatophyte infection is best diagnosed by finding intracellular amastigotes within macrophages on Giemsa stain

Q106. All of the following statements are true EXCEPT:

- a) Local anesthetics block voltage-gated sodium channels, preventing propagation of action potentials
- b) Bupivacaine is more cardiotoxic than lidocaine because of strong binding to cardiac sodium channels
- c) Intravenous lipid emulsion is used in severe local anesthetic systemic toxicity, especially with cardiovascular instability
- d) Epinephrine-containing test dose completely eliminates the risk of intravascular local anesthetic injection

Q107. All of the following statements are true EXCEPT:

- a) Spinal anesthesia can cause hypotension by blocking sympathetic vasoconstrictor fibers, reducing systemic vascular resistance and venous return
- b) High spinal block can reduce cardioaccelerator sympathetic outflow from T1–T4, contributing to bradycardia
- c) Nausea during spinal anesthesia may occur from hypotension and reduced cerebral perfusion



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- d) Bradycardia after high spinal anesthesia is primarily due to direct stimulation of beta-1 receptors by the intrathecal local anesthetic

Q108. All of the following statements are true EXCEPT:

- a) Bowel obstruction and recent oral intake increase aspiration risk during induction
- b) Preoxygenation increases oxygen reserve and prolongs safe apnea time
- c) Rapid sequence induction aims to minimize time between loss of airway reflexes and tracheal intubation
- d) Routine prolonged mask ventilation after induction is preferred in all full-stomach patients because it decreases gastric insufflation and aspiration risk

Q109. All of the following statements are true EXCEPT:

- a) Succinylcholine is a depolarizing neuromuscular blocker that initially may cause fasciculations
- b) Severe burns, denervation injuries, prolonged immobilization, and certain myopathies increase risk of succinylcholine-induced hyperkalemia
- c) Succinylcholine is useful for rapid sequence induction because of rapid onset and short duration in typical patients
- d) Succinylcholine prevents hyperkalemia in denervated muscle by competitively blocking acetylcholine release from presynaptic terminals

Q110. A 26-year-old woman remains apneic and paralyzed for 90 minutes after receiving succinylcholine during an otherwise uncomplicated procedure. She is awake but unable to move, and train-of-four monitoring shows persistent neuromuscular blockade. Family history reveals an uncle who “could not wake up” after anesthesia. Dibucaine number is low.

All of the following statements are true EXCEPT:

- a) Pseudocholinesterase deficiency can prolong succinylcholine paralysis
- b) Management includes mechanical ventilation and sedation until neuromuscular function returns
- c) A low dibucaine number suggests atypical pseudocholinesterase activity
- d) Neostigmine reliably and immediately reverses succinylcholine paralysis in pseudocholinesterase deficiency

Q111. All of the following statements are true EXCEPT:

- a) Opioids depress ventilation primarily by reducing brainstem responsiveness to carbon dioxide



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- b) Naloxone is a competitive opioid receptor antagonist used to reverse life-threatening opioid-induced respiratory depression
- c) Recurrent respiratory depression may occur because naloxone duration can be shorter than that of many opioids
- d) Naloxone permanently inactivates opioid receptors, eliminating the need for further monitoring after a single dose

Q112. All of the following statements are true EXCEPT:

- a) Nitrous oxide diffuses into closed air spaces faster than nitrogen diffuses out, expanding those spaces
- b) Nitrous oxide should be avoided in pneumothorax, bowel obstruction, middle ear surgery, and recent intraocular gas bubble
- c) Nitrous oxide has analgesic properties but is not sufficiently potent as a sole general anesthetic
- d) Nitrous oxide reduces the volume and pressure of trapped gas collections by extracting nitrogen faster than it enters the cavity

Q113. All of the following statements are true EXCEPT:

- a) Pregnancy increases risk of aortocaval compression in the supine position, reducing venous return and uteroplacental perfusion
- b) Neuraxial anesthesia can worsen hypotension by sympathetic blockade and vasodilation
- c) Maternal hypotension can contribute to fetal heart rate abnormalities through reduced uteroplacental blood flow
- d) Supine positioning is preferred after spinal anesthesia because it maximizes venous return by compressing the inferior vena cava

Q114. All of the following statements are true EXCEPT:

- a) Neuromuscular blockers provide paralysis but do not provide hypnosis, amnesia, or analgesia
- b) Hemodynamic instability may lead clinicians to reduce anesthetic depth, increasing awareness risk
- c) Processed EEG monitors may help assess hypnotic depth but do not replace clinical judgment
- d) If a patient is paralyzed, awareness is impossible because lack of movement proves adequate anesthesia

Q115. All of the following statements are true EXCEPT:

- a) Placenta previa classically presents with painless vaginal bleeding in the second half of pregnancy
- b) Prior cesarean delivery with placenta previa increases the risk of placenta accreta spectrum



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- c) Digital vaginal examination should be avoided until placenta previa is excluded
- d) Placenta accreta spectrum is caused by excessive development of a normal decidua basalis separating trophoblast from myometrium

Q116. A physiology experiment records ventricular function curves, showing a shift from curve A to curve b) The new curve demonstrates increased force of contraction at the same preload, indicating enhanced myocardial contractility. The researcher explains that this shift is due to a factor that increases intracellular calcium availability and strengthens cardiac muscle contraction independent of fiber length.

Which of the following is most likely responsible for this shift?

- a) Norepinephrine
- b) Acetylcholine
- c) Intracellular acidosis
- d) Calcium channel blockers

Q117. A 9-year-old boy is referred with a persistent fishy odor emanating from his sweat, urine, saliva, and breath. The odor worsens after consumption of foods rich in choline such as eggs and fish. Physical examination is otherwise normal. Laboratory investigations and genetic testing confirm a metabolic defect where a compound responsible for the odor is not properly oxidized in the liver. The child is managed with dietary modification.

Which enzyme deficiency is responsible for this condition?

- a) Fumarylacetoacetate hydrolase
- b) Methane monooxygenase
- c) Monooxygenase 3 (FMO3)
- d) Amino acid oxidase

Q118. All of the following statements are true EXCEPT:

- a) Absence of an intrauterine pregnancy above the discriminatory β -hCG zone raises concern for ectopic pregnancy
- b) Methotrexate may be used in selected stable patients with unruptured ectopic pregnancy
- c) Tubal rupture can cause hemoperitoneum and shock requiring urgent surgery
- d) Methotrexate is preferred in hemodynamically unstable patients with suspected ruptured ectopic pregnancy because it acts immediately



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Q119. All of the following statements are true EXCEPT:

- a) Shoulder dystocia is suggested by turtle sign and failure of shoulder delivery after the head
- b) McRoberts maneuver and suprapubic pressure are first-line maneuvers
- c) Excessive downward traction may increase risk of brachial plexus injury
- d) Fundal pressure is preferred because it dislodges the anterior shoulder from behind the pubic symphysis

Q120. All of the following statements are true EXCEPT:

- a) Chronic anovulation causes prolonged estrogen stimulation without adequate progesterone opposition
- b) Atypical endometrial hyperplasia is a premalignant lesion associated with increased risk of endometrioid adenocarcinoma
- c) Obesity increases peripheral aromatization of androgens to estrogens, contributing to risk
- d) Continuous progesterone deficiency protects the endometrium by inducing secretory transformation and stromal decidualization

Q121. Match the following kits and their uses under given program:



Syndrome	Kit
A. Lower abdominal pain	1. Grey, kit 1
B. Urethral discharge	2. Green, Kit 2
C. Non herpetic genital ulcer	3. White, kit 3
D. Vaginal discharge	4. Yellow, kit 6

- a) A4, B1, C3, D2
- b) A1, B4, C3, D2
- c) A3, B2, C4, D1
- d) A4, B3, C2, D1

Q122. A 18-month-old child presents to clinic for scheduled vaccination of DPT booster, OPVB, MCV2. The child has mild upper respiratory tract infection with low-grade fever (37.8°C) and runny nose. The mother is worried and asks whether vaccination should be postponed. What is the most appropriate advice?



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- a) Postpone vaccination until complete recovery
- b) Give antibiotics first
- c) Proceed with vaccination
- d) Contraindicated due to fever

Q123. A researcher compares two countries. Country A has high literacy and low infant mortality but low per capita income. Country B has higher income but poorer health indicators. The researcher prefers an index that excludes income and focuses purely on physical well-being. Which index should be used?

Country A



Country B



- a) HDI
- b) PQLI
- c) DALY
- d) GNI

Q124. A national health program prioritizes diseases that cause the highest combined burden of premature death and long-term disability. Conditions like depression and road traffic injuries are ranked high despite relatively low mortality rates. Which measure is most appropriate for this prioritization?



- a) Mortality rate
- b) Incidence rate
- c) DALY



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d) PQLI

Q125. In a district, the incidence of malaria has been significantly reduced through vector control measures, early diagnosis, and treatment. However, new cases continue to occur every year, although at a much lower rate than before. Which level of disease reduction is best described here?

- a) Elimination
- b) Eradication
- c) Control
- d) Extinction

Q126. A wedding function was attended by 300 guests. Within 24 hours, more than 150 attendees developed vomiting and diarrhoea after consuming a contaminated dessert. The number of cases peaked rapidly and declined sharply within the next 48 hours, with no new cases reported afterward. Epidemiological investigation revealed a single exposure to contaminated food. Which type of epidemic best explains this pattern?



- a) Continuous common source epidemic
- b) Propagated epidemic
- c) Point source epidemic
- d) Mixed epidemic

Q127. A researcher identifies 150 patients with carcinoma cervix from a tertiary care hospital. He selects 150 women without the disease from the same population. Both groups are interviewed regarding past exposure to HPV infection and early sexual activity. Which study design is being used?

- a) Cohort study
- b) Case-control study
- c) Cross-sectional study
- d) Randomized trial

Q128. A country experiences a decline in both birth and death rates. Family planning programs are widely accepted, and literacy levels have improved. Population growth continues but at a slower rate compared to earlier decades. Which phase of demographic cycle is this?

- a) High stationary
- b) Early expanding
- c) Late expanding
- d) Declining phase

Q129. All of the following are non-contraceptive benefits of OCP except ?



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- a) Reduction of RTI/STI
- b) Lower chance of Ectopic pregnancy
- c) Risk reduction for Ovarian cancer
- d) Risk reduction for endometrial cancer

Q130. Which of the following indicator shows the efficacy of early detection of leprosy in NLEP?

- a) Annual new case detection rate per lac
- b) Proportion of multi bacillary among new cases
- c) Proportion of treatment completion rates
- d) Proportion of grade 2 disability cases among the general population

Q131. Under NVBDCP, the co-formulation (artemether + lumefantrine) given below available in the fixed dose combination of:



- a) 25 mg and 12.5 mg
- b) 120 mg and 200 mg
- c) 20 mg and 120 mg
- d) 120 mg and 20 mg

Q132. A pregnant female, <34 weeks gestation, reports with mild-moderate anaemia. What is the best management plan as per the Anaemia Mukht Bharat Program?

- a) Two tablets of IFA + Inj. Iron sucrose
- b) 3 tablets of IFA per day
- c) IV Iron sucrose, if female is not tolerating oral IFA
- d) IM Ferric carboxy maltose



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Q133. There is an outbreak of acute encephalitis in the community and a vaccination drive is launched. All of the following is true about the disease given in this condition except ?

- a) Live vaccine, intramuscular
- b) Killed vaccine, intramuscular
- c) Vector- culex mosquito
- d) Pigs- amplifiers

Q134. Which of the following options are correct about VVM?



- 1. VVM has a chemical indicator in the circle, which changes colour
 - 2. VVM gives an idea for number of days for expiry of vaccine
 - 3. It is the only tool that is available at any time in the process of distribution and at the time a vaccine is administered at health centre to check for heat damage
 - 4. VVM tells about the efficacy of the vaccine.
 - 5. The expiry date of the vaccine can be relaxed, if the VVM is intact.
- a) 1, 3 are correct
 - b) 1, 3, 5 are correct
 - c) 3 is correct
 - d) All are correct

Q135. The peripheral district of Kerala was affected by floods and the government distributed some tablets along with some chemicals for disease prophylaxis following flood. Correct match:



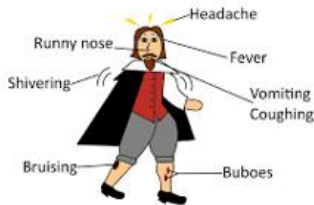
- a) Zinc phosphide- rat, ciprofloxacin- leptospirosis
- b) Malathion- mosquito, doxycycline- leptospirosis
- c) Malathion- mosquito, ciprofloxacin- leptospirosis
- d) Malathion- rat, doxycycline- leptospirosis

Q136. Outbreak of bubos occurs in community. The responsible vector is:



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- a) *Xenopsylla cheopis*
- b) *Phlebotomous argentipes*
- c) *Sarcoptes scabiei*
- d) *Hemophilus ducreyi*

Q137. A female presents with Anogenital warts. Which of the following serotypes has high risk of ca cervix in this female?

- a) hpv 1
- b) HPV 5
- c) HPV 11
- d) HPV 18

Q138. A child born to HIV positive mother is started with cotrimoxazole prophylaxis, all are incorrect about this except?

- a) Started at birth irrespective of HIV status and continued till 18month
- b) Started at 6week of age if child is HIV positive and continued till life long
- c) Started at 6week of age irrespective of HIV status and continued till 18month
- d) Started at 6week of age irrespective of HIV status and continued till 5years

Q139. Anil and Sunil are of 3 and 4years of age. Anil developed fever then rash and treated at home in traditional way only. 10 days later Sunil also had same features. Mother took Sunil to the subcentre and ANM diagnosed it as measles. Which of the following is true ?

ANIL



SUNIL



- a) Anil is primary with index case
- b) Sunil is primary with index case
- c) Anil is secondary with index case
- d) Sunil is secondary with index case



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Q140. An individual reports with a large wound from dog bite which is bleeding. Consider following options:



1. Anti-rabies vaccine, intramuscular – on day 0, 3, 7, 14, 28
2. Clean the wound for 10-15 minutes with soap and water
3. Equine rabies immunoglobulin at the rate of 40 IU/kg
4. Do immediate, proper wound suturing
5. Cover the wound with sterile dressing and tight bandage

Which of the above statements is/are correct ?

- a) 1, 2, 4
- b) 1, 2, 3
- c) 2, 3, 5
- d) All are correct

Q141. Which of the following will best measure to prevent typhoid transmission in community?

- a) Typhoid vaccine administration is best method to prevent transmission.
- b) Hygiene and sanitation control is more important than typhoid vaccine.
- c) Kill mosquitos to prevent disease spread
- d) Drug resistance in typhoid is not big problem as in TB.

Q142. Which of the following is not routinely screened for before blood transfusion?

- a) Malaria
- b) Dengue
- c) HBV
- d) HIV

Q143. A patient presents with black eschar are on the back with chills and high-grade fever. Correct match is:



- a) Epidemic typhus- mite- R. provazaki
- b) Scrub typhus- mite- R. Provazaki
- c) Epidemic typhus- mite- R. tsutsugamushi
- d) Scrub typhus- mite- R. tsutsugamushi

Q144. A man from Chhattisgarh presented with muscle weakness, leg spasms, pure motor weakness, all are correct about this condition except ?



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- a) Toxin responsible is water soluble
- b) More commonly seen in malnourished individuals
- c) Toxin leads to flaccid paralysis
- d) Vitamin C is preventing this condition

Q145. An anganwadi worker takes weight and height of 4-year-old child and finds the height for age <-3 SD. The likely cause is:

- a) Chronic malnutrition
- b) Acute malnutrition
- c) Recent malnutrition
- d) No malnutrition

Q146. Broken glass vial is disposed in:



- a) Yellow bag
- b) Red bag
- c) Blue puncture proof container
- d) White sharps container

Q147. Green colour on triage is:



- a) Dead patients
- b) High priority patients
- c) Ambulatory patients
- d) Moderate priority patients

Q148. Which of the following statement is true?

- a) PPV increases with increasing prevalence
- b) Sensitivity depends on prevalence
- c) Specificity depends on prevalence
- d) In low prevalence, positive of a highly sensitive test means true positive.



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- Q149. The graphical presentation for pass percentage of different schools of Delhi in CBSE 12th board exam is done by which of the following?
- Bar chart
 - Histogram
 - Line diagram
 - Pie chart
- Q150. A study for smoking and lung cancer finds the attributable risk percentage for smoking as 95%. All of the following are correct interpretation except ?
- 95% cases of lung cancer can be avoided in smokers if smoking is eliminated
 - 95% cases of lung cancer can be avoided in general population if smoking is eliminated
 - It is the fraction of people who develop disease among the exposed population
 - It is measured by dividing risk difference by incidence of disease in exposed group

ANSWERS

1	c
2	b
3	b
4	c
5	d
6	a
7	c
8	b
9	b
10	b
11	d
12	a
13	a
14	c
15	d
16	b
17	a
18	a
19	b
20	c
21	b
22	a
23	a
24	a
25	c
26	d

27	b
28	d
29	c
30	b
31	d
32	a
33	d
34	a
35	d
36	b
37	d
38	a
39	d
40	a
41	d
42	b
43	d
44	a
45	d
46	d
47	c
48	b
49	d
50	d
51	d
52	d

53	c
54	b
55	b
56	c
57	c
58	c
59	b
60	b
61	a
62	b
63	a
64	d
65	d
66	b
67	d
68	b
69	d
70	b
71	b
72	a
73	d
74	d
75	d
76	d
77	d
78	c

79	d
80	d
81	c
82	d
83	c
84	d
85	d
86	d
87	d
88	d
89	d
90	d
91	d
92	b
93	d
94	d
95	d
96	c
97	d
98	c
99	d
100	d
101	c
102	d
103	d
104	d

105	d
106	d
107	d
108	d
109	d
110	d
111	d
112	d
113	d
114	d
115	d
116	a
117	c
118	d
119	d
120	d
121	a
122	c
123	b
124	c
125	c
126	c
127	b
128	c
129	a
130	d



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131	c
132	c
133	a
134	c

135	b
136	a
137	d
138	c

139	d
140	b
141	b
142	b

143	d
144	c
145	a
146	c

147	c
148	a
149	a
150	b

